

Second Shift

Turns what this shift said into what the next person and the GP can read.

WHAT IT IS

A **voice-first AI agent for whoever is on the shift**: unpaid family carers and paid support workers in dementia care (home or residential). Speak or type. It logs events on the **person**, spots patterns, writes a **shift handout**, and generates an **evidence-cited GP / memory-clinic brief**. Nurses and doctors get a **simple view** of the same record.

WHO USES IT

Ravi, 34, unpaid, cares for his father (dementia, 6 medicines) and forgets the week at the GP. **Priya**, a support worker, was off last week and must still see that Able vomited after food. A nurse or GP opens the same person and reads the trend. They do not re-enter meals.

WHY IT MATTERS

- ~982,000 people with dementia in the UK
- Staff and family change; the person does not
- Unpaid care is 50% of dementia’s £42bn cost
- Home and shift observations rarely reach the GP

HOW IT WORKS



Speech to text → structured events on the person → rules flag patterns (including “similar last week”) → shift handout (6h or 12h) plus a cited PDF. Family weekend cover can still use a 72-hour preset.

WHAT’S NOVEL

Nourish and Birdie digitise the **paid care record**. Heidi transcribes the **appointment**. Nobody keeps **memory on the person** from natural speech and turns it into a cited brief for the next worker and the GP.

PRODUCT	COVERS	MISSING VS. SECOND SHIFT
Nourish / Birdie / PASSforcare	DSCR, visits, eMAR for paid staff	Operational record, not speech → person memory → cited brief
Jointly / Share2Care	Circle of care, contingency plan	Tap-based organisation, not voice → patterns → GP brief
Heidi / Tortus / Accurx	AI scribe of the GP appointment	Knows nothing about what happened on the last shifts
Curendi / KinKeeper	Guidance or family journal PDF	Not voice-first, not evidence-cited from the live log

Four novelty points

- Speech → next worker and GP
- Memory stays with the person when staff change
- Evidence-cited briefs (every line traces to a log)
- Handover follows the shift; 72h is a family preset

Safety stance

Organises notes and drafts questions. Recurrence banners cite dates, they do not diagnose. Never triages or gives treatment advice. Not a medical device.

THE 30-SECOND DEMO

Ravi: “Gave dad his 8pm meds, 40 minutes late. More confused again this evening, third time this week.” Flag after the dose change. One-page GP brief. Priya: “Able has eaten; after eating he was vomiting.” Banner: similar issue last week, logged by someone else. Nurse/GP simple view.

The next person on shift, and the GP, read evidence instead of starting from zero.

Beachhead: dementia. Family and paid staff. Later: stroke, Parkinson’s, frailty. Synthetic demo data. Not a medical device.